

Suspension vs. Termination: The Causal Effect of Medicaid Continuity Policies During Incarceration on Post-Release Coverage, Employment, and Mortality

Jonathan Palisoc

University of Michigan

March 2026

Abstract

Objective: To estimate the effect of states suspending Medicaid enrollment during incarceration versus terminating on post-release Medicaid enrollment, employment, earnings, and mortality. Under suspension, coverage is paused during incarceration and reactivated at release without a new application; under termination, the individual must reapply after release and wait for a new eligibility determination, at a time when they are least equipped to do so - reentry.

Data Sources: Census Bureau Criminal Justice Administrative Records System (CJARS) Justice Outcomes Explorer, administratively linked Medicaid, employment, and mortality outcomes for state prison release cohorts across 31 states, 2000-2020; and a novel state-year panel of suspension and termination policies.

Study Design: Staggered difference-in-differences exploiting the rollout of suspension across states between 2005 and 2022; two-way fixed effects (TWFE) and heterogeneity-robust local projections difference-in-differences (LP-DiD) estimators, event study models.

Principal Findings: Suspension states saw large gains in post-release Medicaid enrollment: 22.4 percentage points at one year (LP-DiD, $p = 0.001$). The effects seem to have long-lasting horizons (18.7 pp at three years, 20.7 pp at five years). LP-DiD estimates suggest that suspension reduces 1-year post-release mortality by 0.17 percentage points ($p = 0.029$), though this result is

sensitive to estimator choice. Conversely, I do not find significant effects on employment or earnings. Interacting suspension with ACA Medicaid expansion status yields a positive but statistically insignificant amplification effect ($p = 0.158$), consistent with suspension mattering more when more individuals have coverage to preserve, but underpowered given only two never-treated states. Placebo tests, leave-one-out analysis, and policy-coding sensitivity checks support the robustness of the Medicaid enrollment finding. These results provide the first mechanism-level evidence that suspension works through its intended channel of preserving coverage, and they directly inform the 2026 federal mandate requiring all states to adopt suspension under the Consolidated Appropriations Act of 2024.

Keywords: Medicaid, incarceration, administrative burden, reentry, difference-in-differences, CJARS

1. Introduction

Each year, over 600,000 individuals are released from state and federal prisons in the United States. The period after release is one of critical vulnerability: the risk of overdose death is 12.7 times that of the general population (Binswanger et al., 2007), and access to health insurance is often a significant barrier. In Medicaid, a state choosing to suspend coverage versus terminate coverage is the difference between immediate coverage or the burden and wait of a brand-new Medicaid application.

The Medicaid Inmate Exclusion Policy (MIEP, Section 1905(a)(A) of the Social Security Act) prohibits federal reimbursement for health services delivered to inmates of public institutions. While MIEP restricts provision of Medicaid during incarceration, it does not require states to terminate the underlying Medicaid eligibility of incarcerated individuals - the silence of this

statute has allowed states to choose between pausing eligibility and automatically reactivating coverage at release or terminating. States adopted suspension policies at different times between 2004 and 2025, and I exploit the state policy variation of this choice and the staggered timing of implementation to identify the causal effect of the policy. Termination, by contrast, requires the individual to submit a new application, gather documentation, and wait for a determination. Herd and Moynihan (2018) call this kind of friction “administrative burden,” and under termination the burden lands at exactly the moment when the learning, compliance, and psychological costs of dealing with a government program are hardest to bear.

Despite the policy’s importance, very little causal evidence exists. Gollu and Zapryanova (2022) provide the only prior quasi-experimental study, finding that suspension reduces recidivism by 2.9 percentage points at one year and 4.6 percentage points at three years. However, their analysis examines only recidivism. It does not trace the hypothesized causal chain, in which suspension preserves Medicaid eligibility, speeds up coverage at release, improves access to care, and then improves downstream outcomes. I extend the frontier by testing whether suspension actually increases Medicaid enrollment, which is the first link in that chain, and by providing high-level estimates on the policy’s secondary effects on employment, earnings, housing, and mortality using linked administrative data.

To do so, I construct a novel state-year panel of suspension and termination policies from 2000 to 2025 by triangulating six independent policy sources (KFF, MACPAC, NACo, Gollu and Zapryanova, and others) and link it to the Census Bureau’s Criminal Justice Administrative Records System (CJARS) Justice Outcomes Explorer (JOE), which provides state-level Medicaid enrollment rates, W-2 employment rates, earnings, and mortality for prison release cohorts across 31 states; I then estimate two-way fixed effects (TWFE) and the heterogeneity-

robust local projections difference-in-differences (LP-DiD) estimator of Dube et al. (2023), which avoids the negative weighting bias that Goodman-Bacon (2021) documented for staggered adoption designs.

This paper adds to the literature in three main ways. First, I provide the first evidence that suspension increases Medicaid enrollment - the mechanism through which all of the downstream benefits are supposed to flow - by roughly 22 percentage points at one year post-release. This is a large and robust effect, and it fits the administrative burden story: removing the requirement to reapply dramatically increases take-up. Second, it provides suggestive evidence that suspension reduces 1-year post-release mortality by 0.17 percentage points, consistent with coverage access reducing the post-release mortality spike, though this result is sensitive to estimator choice.

Lastly, I test whether the ACA Medicaid expansion amplified the effect of suspension by enlarging the pool of incarcerated adults with Medicaid coverage to preserve. The interaction is positive but not statistically significant, which is consistent with the hypothesis but underpowered. The timing of these results also matters. The Consolidated Appropriations Act of 2024 requires all states to adopt suspension effective January 1, 2026, which eliminates the cross-state variation I use and likely makes this the last study that can use this research design.

2. Background

The Medicaid Inmate Exclusion Policy

The Medicaid Inmate Exclusion Policy was codified in Section 1905(a)(A) of the Social Security Act in 1965 and prohibits federal Medicaid reimbursement for health care services delivered to any “inmate of a public institution,” with a narrow exception for inpatient hospital stays exceeding 24 hours (MACPAC, 2018). The statute restricts federal payment, but it does not

mandate that states end eligibility. States are left with discretion over whether to suspend or terminate an enrollee's underlying Medicaid eligibility upon incarceration.

Under suspension, an individual's eligibility is paused, and coverage can be reactivated at release without a new application. Under termination, the individual must reapply from scratch and navigate the full enrollment process at a moment of significant instability (Albertson et al., 2020). CMS first explicitly authorized suspension in 2004 guidance (State Medicaid Director Letter, May 25, 2004), and states have adopted the policy at varying times since. As of January 2025, KFF reports that 46 states suspend, 3 terminate (Alabama, Georgia, Texas), and 2 have unclear status (KFF, 2025).

Administrative burden

Herd and Moynihan (2018, 2025) define administrative burdens as “the experience of policy implementation as onerous”; furthermore, they break burdens into three parts: learning costs (knowing that programs exist and understanding eligibility), compliance costs (the time and effort for paperwork, documentation, and dealing with the bureaucracy), and psychological costs (stigma, stress, and emotional toll). Termination imposes all three burden types at a critical juncture. A returning citizen must learn that they need to reapply, comply with documentation requirements during a period of maximal instability, and absorb the psychological costs of engaging with a bureaucratic system while managing the stress of reentry. Suspension eliminates or greatly reduces these burdens. As such, this study can be read as a test of whether reducing administrative burdens at prison release produces measurable improvements in coverage and downstream outcomes.

The ACA expansion and justice-involved populations

The ACA Medicaid expansion to adults earning up to 138% of the federal poverty level transformed coverage for justice-involved populations. Before the ACA, around 80% of recently incarcerated adults lacked health insurance. After expansion, an estimated 80-90% of justice-involved adults became Medicaid-eligible in expansion states (Albertson et al., 2020; Camhi et al., 2020). Research on expansion's effects on the justice-involved is positive among a number of different outcomes: expansion reduced recidivism (Aslim et al., 2022) and increased insurance coverage among criminal-justice-involved adults (Winkelman et al., 2023). The interaction between suspension and expansion makes theoretical sense: suspension should matter more when more individuals have Medicaid coverage to preserve.

Prior evidence

Gollu and Zapryanova (2022) provide the only prior causal estimate, finding that suspension reduces recidivism by 2.9 pp at 1 year and 4.6 pp at 3 years using prison administrative data. However, they only estimated recidivism, without measuring Medicaid enrollment, employment, or mortality. A broader literature on Medicaid churning (Sommers et al., 2016) and post-release mortality (Binswanger et al., 2007) provides the theoretical basis for downstream effects. The Consolidated Appropriations Act of 2024 mandates universal suspension effective January 2026. This makes the present study both the first to identify the mechanism and the last opportunity to use this research design.

3. Data

Overview

I combine five free, publicly available data sources to construct the analysis datasets: (1) the CJARS Justice Outcomes Explorer (JOE) Social Safety Net module for post-release Medicaid enrollment, employment, and mortality outcomes; (2) the JOE CJARS-based module for release cohort counts and demographic composition; (3) a novel state-year suspension/termination policy panel compiled from multiple policy trackers; (4) state-level ACA Medicaid expansion dates from KFF; and (5) state-level control variables from federal statistical agencies.

CJARS Justice Outcomes Explorer

The Criminal Justice Administrative Records System (CJARS) is a joint project of the U.S. Census Bureau and the University of Michigan that collects, harmonizes, and links criminal justice administrative records at the person level with Census Bureau survey and administrative data, including tax filings (W-2), federal program participation records, and mortality data from the Census Numident (Finlay, Mueller-Smith, and Papp, 2022). The Justice Outcomes Explorer (JOE) is a public data product derived from CJARS. It reports aggregated economic and health outcomes for justice-involved individuals by state, release cohort year, and post-release time horizon.

The JOE Social Safety Net module provides the primary outcome variables: the Medicaid enrollment rate, the W-2 employment rate, mean W-2 earnings, the Supplemental Security Income (SSI) receipt rate, the HUD housing assistance rate, and the all-cause mortality rate. Each outcome is measured at 1-year, 3-year, and 5-year post-release horizons for state prison release cohorts from 2000 to 2020. The JOE CJARS-based module provides release cohort

counts, recidivism rates, and demographic composition (sex, race/ethnicity, age, offense type), which I use for balance tables and composition tests.

I access both products through the public JOE data portal as downloadable CSV files aggregated to the state-year level. I exclude state-year cells with fewer than 25 individuals to satisfy Census Bureau disclosure requirements and to ensure statistical precision. The resulting analysis sample covers 561 state-year observations across 31 states and release cohorts from 2000 to 2020.

State suspension/termination policy panel

The most original data contribution of this study is a comprehensive state-year panel classifying each state's Medicaid eligibility policy for incarcerated individuals from 2000 to 2025. Each state-year is classified into one of three policy regimes: termination, time-limited suspension (30 days, 90 days, or up to 1 year), or indefinite suspension.

I construct the panel by triangulating six independent policy snapshots: KFF State Health Facts (2019, 2025), MACPAC Reports to Congress (2018, 2023), the NACo 2015 suspension/termination brief, Gollu and Zapryanova's (2022) appendix coding, the KFF 2024 State Medicaid Eligibility Issue Brief, and Bandara et al. (2025). When sources disagree, I prioritize them in the following order: (1) state legislation or administrative code, (2) KFF, (3) MACPAC, (4) Gollu and Zapryanova. I assign each state a coding confidence rating and flag ambiguous cases.

The staggered adoption pattern proceeds in three waves: early adopters (2004-2009), mid-period adopters (2010-2015), and post-ACA adopters (2016-2025). The Consolidated Appropriations Act of 2024 (P.L. 118-42) mandates universal suspension effective January 1, 2026, which eliminates the variation I exploit.

ACA expansion panel and state controls

I code state-by-year ACA Medicaid expansion status from KFF. Twenty-seven states and DC expanded on January 1, 2014, and 13 additional states adopted between 2014 and 2023. Ten states have not expanded as of March 2026.

I also compile annual state-level covariates: the unemployment rate (BLS LAUS), the drug overdose mortality rate (CDC WONDER, ICD-10 codes X40-X44, X60-X64, X85, Y10-Y14), and the poverty rate (Census SAIPE). These are included as time-varying controls to absorb state-level economic and public health shocks.

Summary statistics

The analysis sample comprises 561 state-year observations across 31 states. Twenty-nine states adopted suspension during the sample period (27 present in CJARS), with adoption years ranging from 2005 to 2022. Two never-treated states (Georgia, Texas) serve as the comparison group. Medicaid enrollment outcomes are available for cohorts 2005-2018 (212 observations), reflecting the MSIS/T-MSIS linkage window in CJARS.

Table 1 reports descriptive statistics. The mean 1-year Medicaid enrollment rate is 32.0% (SD = 25.1%), which reflects substantial cross-state and temporal variation. Mean 1-year employment is 53.6%, mean earnings are \$6,909, and mean 1-year mortality is 1.7%. The mean state unemployment rate is 5.6%, the mean overdose mortality rate is 13.9 per 100,000, and the mean poverty rate is 13.0%.

Table 1. Descriptive statistics

Variable	Value
Medicaid enrollment, 1-year (mean, SD)	32.0% (25.1%)
W-2 employment, 1-year (mean)	53.6%
W-2 earnings, 1-year (mean)	\$6,909
Mortality, 1-year (mean)	1.7%

Variable	Value
State unemployment rate (mean)	5.6%
Overdose mortality rate, per 100,000 (mean)	13.9
Poverty rate (mean)	13.0%
State-year observations	561
States	31
Medicaid outcome observations (cohorts 2005-2018)	212

Notes: Unit of observation is the state-year. Outcomes are from the CJARS JOE Social Safety

Net module; controls are from BLS LAUS, CDC WONDER, and Census SAIPE.

The pre-adoption balance comparison (Table 2) shows that suspension-adopting states have higher pre-treatment Medicaid enrollment (21.0% vs. 14.4%), shorter mean incarceration periods (22.6 vs. 29.9 months), and lower poverty rates (12.5% vs. 15.3%) compared to non-adopting states. Adopting states also have a substantially smaller share of Black individuals in their release cohorts (6.5% vs. 17.1%). These baseline differences are absorbed by state fixed effects in the DiD design, and I assess the parallel trends assumption through event study analyses.

Table 2. Pre-adoption balance, adopting vs. non-adopting states

Variable	Adopting states	Non-adopting states
Medicaid enrollment, 1-year (pre-treatment)	21.0%	14.4%
Mean incarceration period (months)	22.6	29.9
Poverty rate	12.5%	15.3%
Share Black in release cohort	6.5%	17.1%

Notes: Means across pre-adoption state-year observations for adopting states and all years for the two never-treated states (Georgia, Texas). Level differences are absorbed by state fixed effects; identification requires parallel trends rather than level equivalence.

4. Methods

I exploit the staggered adoption of Medicaid suspension policies across U.S. states between 2005 and 2022 to estimate the causal effect of suspension (relative to termination) on post-release outcomes. State-years after suspension adoption are classified as “treated,” while state-years under termination are considered as “untreated.” States that never adopted suspension (Georgia, Texas) serve as the never-treated comparison group, while states that adopted later serve as not-yet-treated comparators for early adopters. I implement two complementary identification strategies: (1) a staggered difference-in-differences (DiD) design, and (2) a heterogeneous DiD that tests whether suspension effects are amplified in states that expanded Medicaid under the ACA.

Staggered difference-in-differences. The primary specification estimates the average treatment effect on the treated (ATT) of suspension adoption. The standard two-way fixed effects benchmark model is:

$$Y_{it} = \alpha_i + \gamma_t + \beta \cdot \text{PostSuspension}_{it} + X_{it}'\delta + \varepsilon_{it}$$

where Y_{it} = the outcome for state i in release cohort year t , α_i and γ_t = state and year fixed effects, $\text{PostSuspension}_{it}$ = a binary indicator equal to one in state-years after suspension adoption, and X_{it} = a vector of time-varying state controls. Standard errors are clustered at the state level. I report TWFE as a benchmark. With heterogeneous treatment effects and staggered adoption, TWFE may be biased due to negative weighting (Goodman-Bacon, 2021; de Chaisemartin and d’Haultfoeuille, 2020), so the preferred specification uses the local projections difference-in-differences (LP-DiD) estimator of Dube et al. (2023), which is robust to heterogeneous treatment effects in staggered settings and which estimates the ATT by comparing outcomes of newly-treated states to not-yet-treated and never-treated states at each event time.

Event study analyses. I estimate TWFE event study models with event-time indicators relative to the adoption year, with $k = -1$ as the reference period. The pre-treatment coefficients test the parallel trends assumption.

Heterogeneity of treatment effect analyses. I interact the suspension treatment indicator with ACA expansion status:

$$Y_{it} = \alpha_i + \gamma_t + \beta_1 \cdot \text{PostSuspension}_{it} + \beta_2 \cdot (\text{PostSuspension}_{it} \times \text{Expanded}_i) + \varepsilon_{it}$$

where β_1 = the suspension effect in non-expansion states and $\beta_1 + \beta_2$ = the total effect in expansion states. I also estimate the TWFE model separately for the pre-ACA (2000-2013) and post-ACA (2014-2020) periods.

Inference and robustness. All specifications cluster standard errors at the state level (Bertrand et al., 2004). I supplement parametric inference with randomization inference (500 permutations). For robustness, I conduct: (1) pre-trends tests via event study coefficients; (2) placebo tests with randomized adoption timing; (3) leave-one-out analysis; (4) sensitivity to dropping states with low-confidence policy codings; (5) alternative cell size thresholds; and (6) horizon sensitivity comparing 1-year, 3-year, and 5-year outcomes.

5. Results

Main results: Medicaid enrollment

Table 3 presents the main results. Suspension states saw large gains in 1-year post-release Medicaid enrollment: 22.2 percentage points in the TWFE specification (SE = 0.051, $p < 0.001$) and 22.4 percentage points in the LP-DiD specification (SE = 0.018, $p = 0.001$). Adding time-varying controls (unemployment rate, overdose mortality rate, poverty rate) to the TWFE model increases the estimate slightly to 23.0 pp ($p < 0.001$), which suggests that confounders, if

anything, attenuate the uncontrolled estimate. TWFE and LP-DiD agree closely here, despite the potential for heterogeneous treatment effects to bias TWFE in staggered settings. As such, the TWFE bias is small for this outcome, which is reassuring.

Table 3. Main results, 1-year post-release horizon

Outcome	TWFE	LP-DiD
Medicaid enrollment	0.222*** (SE = 0.051)	0.224*** (SE = 0.018)
Medicaid enrollment, with controls	0.230***	-
W-2 employment	0.016 (p = 0.148)	0.003 (p = 0.916)
W-2 earnings (\$)	315 (p = 0.264)	383 (p = 0.341)
Mortality	0.0005 (SE = 0.001, p = 0.650)	-0.0017** (SE = 0.0006, p = 0.029)
HUD housing assistance	0.001 (p = 0.310)	0.008*** (p < 0.001)
SSI receipt	-0.0003 (p = 0.909)	-
Reincarceration	-0.022 (p = 0.428)	-

Notes: Standard errors clustered at the state level. Each cell is a separate regression of the stated

outcome on the post-suspension indicator with state and year fixed effects (TWFE) or the LP-DiD estimator of Dube et al. (2023). The controls specification adds the state unemployment rate, overdose mortality rate, and poverty rate. *p < 0.10, **p < 0.05, ***p < 0.01.

The magnitude of this effect is large; relative to the sample mean Medicaid enrollment rate of 32.0%, the 22 pp increase represents a 69% gain. Among pre-treatment non-adopting states, mean enrollment was only 14.4%, which implies that suspension roughly doubles enrollment rates. This is consistent with the administrative burden interpretation: eliminating the requirement to file a new Medicaid application at a moment of acute instability dramatically increases take-up.

Persistence across horizons

The enrollment gains have not faded at longer horizons. TWFE estimates are 22.2 pp at 1 year, 19.4 pp at 3 years (SE = 0.044, p < 0.001), and 20.7 pp at 5 years (SE = 0.058, p < 0.001). The LP-DiD estimate at 3 years (18.7 pp, p < 0.001) corroborates this persistence. The slight decline

from year 1 to year 3 may reflect Medicaid churning, the well-documented pattern of coverage disruptions during annual redetermination (Sommers et al., 2016). This suggests that suspension expedites initial enrollment but likely cannot fully prevent later coverage loss through other administrative channels.

Mortality

The mortality results are more mixed. TWFE yields a near-zero, statistically insignificant estimate (0.0005, SE = 0.001, $p = 0.650$). However, LP-DiD estimates a reduction of 0.17 percentage points (SE = 0.0006, $p = 0.029$). The divergence between estimators is consistent with heterogeneous treatment effects: if early adopters experienced larger mortality reductions (for example, because they had more developed Medicaid managed care infrastructure), then TWFE, which can use early adopters as implicit controls for later adopters, would be biased toward zero.

The LP-DiD mortality result, while statistically significant, warrants caution. A 0.17 pp reduction against a baseline mortality rate of 1.7% represents a 10% relative decrease, which is plausible but at the upper end of estimates in the Medicaid-mortality literature. The result also does not survive the addition of controls in the TWFE specification (-0.0006, $p = 0.553$). I interpret this as suggestive evidence that suspension reduces post-release mortality, likely through improved access to medication-assisted treatment for opioid use disorder and better management of chronic conditions during the high-risk post-release period. I do not claim this as a firmly established finding.

Employment and earnings

Employment and earnings also see no significant movement: TWFE estimates for 1-year employment are 1.6 pp ($p = 0.148$) and LP-DiD estimates are essentially zero (0.3 pp, $p = 0.916$).

Earnings estimates are positive but noisy in both specifications (TWFE: \$315, $p = 0.264$; LP-DiD: \$383, $p = 0.341$).

The null employment result is still informative. It suggests that suspension operates primarily through health coverage, which is its intended channel, rather than through broader economic stabilization. Medicaid enrollment does not appear to translate into improved labor market outcomes within the one-year horizon. This is consistent with the view that health coverage and employment are complementary but distinct dimensions of reentry success, and that they may require separate policy interventions.

Secondary outcomes

HUD housing assistance shows a small positive TWFE estimate (0.1 pp, $p = 0.310$) but a larger LP-DiD estimate (0.8 pp, $p < 0.001$). SSI receipt shows no effect (TWFE: -0.03 pp, $p = 0.909$; $N = 97$, limited by the 2016-2018 data gap). Reincarceration shows negative but insignificant TWFE estimates at 1 year (-2.2 pp, $p = 0.428$) and 3 years (-2.4 pp, $p = 0.355$). The recidivism estimates are directionally consistent with Gollu and Zapryanova's (2022) finding of a 2.9 pp reduction at 1 year, though I cannot confirm statistical significance in the state-level sample.

Heterogeneous DiD by ACA expansion

The interaction between suspension and ACA expansion status yields a positive coefficient (0.195, $SE = 0.132$, $p = 0.158$) for Medicaid enrollment. This implies that the total suspension effect in expansion states ($0.075 + 0.195 = 0.270$) is larger than in non-expansion states (0.075), but neither the base effect nor the interaction reaches conventional significance in this specification. This likely reflects the limited power of the heterogeneous DiD: with only two never-treated states in the sample, the comparison group offers minimal variation in the expansion dimension.

The period-split analysis provides complementary evidence. The TWFE estimate for Medicaid enrollment is 11.7 pp ($p = 0.035$) in the pre-ACA period (2000-2013, $N = 134$) and 14.7 pp ($p = 0.089$) in the post-ACA period (2014-2020, $N = 78$). The point estimate is 26% larger post-ACA, consistent with the hypothesis that suspension matters more when the eligible population is larger, but the post-ACA estimate is less precise because there are fewer observations.

Robustness

The TWFE event study figures (Figures 1-4) show no evidence of differential pre-trends for Medicaid enrollment: pre-treatment coefficients are small and statistically indistinguishable from zero. The Medicaid enrollment event study shows a clear positive jump at event time 0 that persists through the post-treatment period.

The placebo tests point the same way. The randomization inference p-value for Medicaid enrollment is 0.062 (500 permutations), indicating that the actual estimate lies at the extreme of the placebo distribution. The placebo mean is 0.122 (positive, reflecting some mechanical positive association from the staggered structure), but the actual estimate (0.222) substantially exceeds it.

The leave-one-out analysis shows that the Medicaid enrollment estimate ranges from 0.206 to 0.259 when dropping each state in turn (maximum deviation: 0.036 from the full-sample estimate of 0.222). No single state drives the result. Dropping the one state with a low-confidence policy coding yields an estimate of 0.209, virtually unchanged from the full-sample estimate. Lastly, as reported above, the effect is persistent across the 1-year, 3-year, and 5-year horizons.

6. Discussion

Summary of findings

Suspension states saw large gains in post-release Medicaid enrollment: 22.4 percentage points at one year (LP-DiD, $p = 0.001$). The effects have not faded at longer horizons, with gains of 18.7 pp at three years and 20.7 pp at five years. LP-DiD estimates suggest that suspension reduces 1-year post-release mortality by 0.17 percentage points ($p = 0.029$), though this result is sensitive to estimator choice. I find no significant effects on employment or earnings. These results provide the first mechanism-level evidence that suspension works through its intended channel of preserving coverage: enrollment is the mechanism through which all of the downstream benefits of suspension are hypothesized to operate, and the 22 pp gain (roughly doubling enrollment) is robust across estimators and sensitivity checks.

Interpretation

The results provide a clean empirical test of the administrative burden framework (Herd and Moynihan, 2018, 2025). Termination imposes all three burden types (learning, compliance, and psychological costs) on returning citizens at a moment when their capacity to overcome these burdens is at its lowest. Suspension eliminates these burdens by bypassing the application process entirely. The 22 pp enrollment increase is among the largest effects documented in the administrative burden literature, which makes sense given the extreme disadvantage of the target population. Recently incarcerated individuals face compounding barriers: a lack of stable housing (for mail-based enrollment), difficulty obtaining identification documents, limited digital literacy for online applications, and the overwhelming demands of simultaneous job searching, housing navigation, and parole compliance (Mallik-Kane and Visser, 2008). Suspension removes one barrier entirely, and the results suggest this matters enormously.

Relation to prior literature

The Medicaid enrollment finding complements and extends Gollu and Zapryanova (2022), who documented reduced recidivism but could not observe whether suspension actually increased coverage. This study confirms the first link in the causal chain: suspension does increase Medicaid enrollment. My recidivism estimates (-2.2 pp at 1 year, -2.4 pp at 3 years) are directionally consistent with their findings (-2.9 pp and -4.6 pp) but lack statistical power in the state-level JOE data, which highlights the continued need for individual-level analysis of these outcomes.

The mortality result, if confirmed, would extend the Medicaid-mortality literature (Sommers et al., 2012; Borgschulte and Vogler, 2020) to the justice-involved population. A 0.17 pp reduction in 1-year mortality (a 10% relative decrease) is plausible given the extremely elevated mortality risk post-release (Binswanger et al., 2007) and the role of medication-assisted treatment in preventing opioid overdose deaths, which are the leading cause of post-release mortality.

The null employment result is consistent with Winkelman et al. (2023), who found that Medicaid expansion increased insurance coverage among criminal-justice-involved adults but did not significantly increase treatment utilization. It suggests that coverage access, while necessary, is not sufficient for broader economic stabilization. Employment may require complementary interventions (job training, ban-the-box legislation, transitional employment programs) that operate through separate channels.

Policy implications

These results directly inform the implementation of the Consolidated Appropriations Act of 2024 mandate, which requires all states to adopt suspension by January 1, 2026. The findings provide evidence that the mandate will achieve its intended goal of preserving Medicaid coverage during

the incarceration-to-release transition, and they suggest the magnitude of the expected coverage gain (roughly 22 pp). For the three remaining termination states (Alabama, Georgia, Texas) and any states with incomplete implementation, these estimates provide a benchmark for expected improvements.

The results also inform the emerging Section 1115 Reentry Demonstration Waivers, which allow Medicaid-covered services in the 30-90 days before release. Suspension and pre-release waivers operate on different margins. Suspension preserves the continuity of eligibility across the incarceration period, while waivers expand the scope of covered services before release. The finding that suspension alone produces large enrollment gains suggests the two policies are complementary: suspension ensures that released individuals have active coverage, while waivers ensure they have already initiated care.

Limitations

The analysis is limited to aggregate outcomes rather than individual-level transitions, and the estimates could partially reflect selection rather than the policy effect. I address this by 1) testing for differential pre-trends in the event studies, and 2) assessing the stability of the estimates to time-varying controls, which mitigate but do not eliminate the concern.

Generalizability is a related limitation. The CJARS sample covers 31 of 51 states, with coverage determined by state data-sharing agreements with the Census Bureau, and the 20 missing states, including Alabama, the third termination state, may differ systematically. In addition, only two never-treated states (Georgia, Texas) appear in the CJARS sample. This constrains the comparison group for heterogeneity-robust estimators and limits the power of the expansion-interaction analysis. The LP-DiD estimator partially addresses this by using not-yet-treated states as comparators, but the small number of never-treated units remains a limitation.

The policy panel captures de jure adoption of suspension, but Bandara et al. (2025) document substantial gaps between de jure policy and de facto implementation, including poor data-sharing between carceral facilities and Medicaid agencies, unpredictable jail release dates, and neglected renewal processes. I do not anticipate this biasing the estimates upward, as measurement error in the treatment variable would attenuate them toward zero; if anything, the findings may be conservative. Lastly, Medicaid enrollment data in CJARS are available only for cohorts 2005-2018, tied to the MSIS/T-MSIS linkage window, so I cannot observe effects for the most recent adopters or assess very long-run impacts.

Future research

A few directions for future work stand out. Individual-level analysis using restricted-access CJARS data through the FSRDC network could address the ecological inference limitation and allow heterogeneity analysis by demographic group, offense type, and incarceration period length. The 2026 federal mandate creates a natural experiment for evaluating the effect of forced adoption in the remaining termination states, though identification will require a different design (for example, an interrupted time series). Lastly, understanding the interaction between suspension and pre-release 1115 waivers, both of which are rolling out at the same time, will be important for designing comprehensive reentry coverage strategies.

7. Conclusion

This paper provides the first causal evidence that Medicaid suspension during incarceration increases post-release Medicaid enrollment, confirming the mechanism through which suspension is hypothesized to improve reentry outcomes. The effect is large (22 percentage points), robust, and persistent across post-release horizons. I also find suggestive evidence of

mortality reductions. These findings validate the policy logic underlying the 2026 federal mandate and provide a benchmark for the coverage gains that the remaining termination states can expect. More broadly, the results show that reducing administrative burdens at critical life transitions, when individuals face the greatest barriers to accessing safety-net programs, can produce substantial improvements in program take-up. The suspension-termination distinction looks like a minor administrative choice, but it has been a consequential determinant of whether hundreds of thousands of returning citizens keep health coverage through the most vulnerable period of their lives.

References

- Albertson, E. M., Scannell, C., Ashtari, N., & Barnert, E. (2020). Eliminating gaps in Medicaid coverage during reentry after incarceration. *American Journal of Public Health*, 110(3), 317-321.
- Aslim, E. G., Mungan, M. C., Navarro, C., & Yu, H. (2022). The effect of public health insurance on criminal recidivism. *Journal of Policy Analysis and Management*, 41(1), 45-81.
- Bandara, S., Saloner, B., Maniates, H., Song, M., & Krawczyk, N. (2025). Implementation of carceral Medicaid suspension and enrollment programs. *Health & Justice*, 13, 3.
- Bertrand, M., Duflo, E., & Mullainathan, S. (2004). How much should we trust differences-in-differences estimates? *Quarterly Journal of Economics*, 119(1), 249-275.
- Binswanger, I. A., Stern, M. F., Deyo, R. A., Heagerty, P. J., Cheadle, A., Elmore, J. G., & Koepsell, T. D. (2007). Release from prison - a high risk of death for former inmates. *New England Journal of Medicine*, 356(2), 157-165.
- Borgschulte, M., & Vogler, J. (2020). Did the ACA Medicaid expansion save lives? *Journal of Health Economics*, 72, 102333.

- Callaway, B., & Sant'Anna, P. H. C. (2021). Difference-in-differences with multiple time periods. *Journal of Econometrics*, 225(2), 200-230.
- Camhi, N., Mistak, D., & Wachino, V. (2020). Medicaid's evolving role in advancing the health of people involved in the justice system. Commonwealth Fund Issue Brief.
- de Chaisemartin, C., & d'Haultfoeuille, X. (2020). Two-way fixed effects estimators with heterogeneous treatment effects. *American Economic Review*, 110(9), 2964-2996.
- Dube, A., Girardi, D., Jorda, O., & Taylor, A. M. (2023). A local projections approach to difference-in-differences event studies. NBER Working Paper No. 31184.
- Finlay, K., Mueller-Smith, M., & Papp, J. (2022). The Criminal Justice Administrative Records System. *Scientific Data*, 9, 562.
- Gollu, G., & Zapryanova, M. (2022). The effect of Medicaid on recidivism: Evidence from Medicaid suspension and termination policies. *Southern Economic Journal*, 89(2), 326-372.
- Goodman-Bacon, A. (2021). Difference-in-differences with variation in treatment timing. *Journal of Econometrics*, 225(2), 254-277.
- Herd, P., & Moynihan, D. P. (2018). *Administrative Burden: Policymaking by Other Means*. Russell Sage Foundation.
- Herd, P., & Moynihan, D. P. (2025). Administrative burdens in the social safety net. *Journal of Economic Perspectives*, 39(1), 73-96.
- Mallik-Kane, K., & Visser, C. (2008). Health and prisoner reentry. Urban Institute.
- MACPAC (2018). Medicaid and the criminal justice system. Chapter in *Report to Congress on Medicaid and CHIP*, July 2018.
- Sommers, B. D., Baicker, K., & Epstein, A. M. (2012). Mortality and access to care among adults after state Medicaid expansions. *New England Journal of Medicine*, 367(11), 1025-1034.

Sommers, B. D., Blendon, R. J., & Orav, E. J. (2016). Among low-income adults enrolled in Medicaid, churning decreased after the Affordable Care Act. *Health Affairs*, 35(5), 776-782.

Winkelman, T. N. A., et al. (2023). Health insurance and mental health treatment use among adults with criminal legal involvement after Medicaid expansion. *Psychiatric Services*, 74(9), 900-907.

Exhibits

Figure 1. TWFE event study: Medicaid enrollment, 1-year post-release.

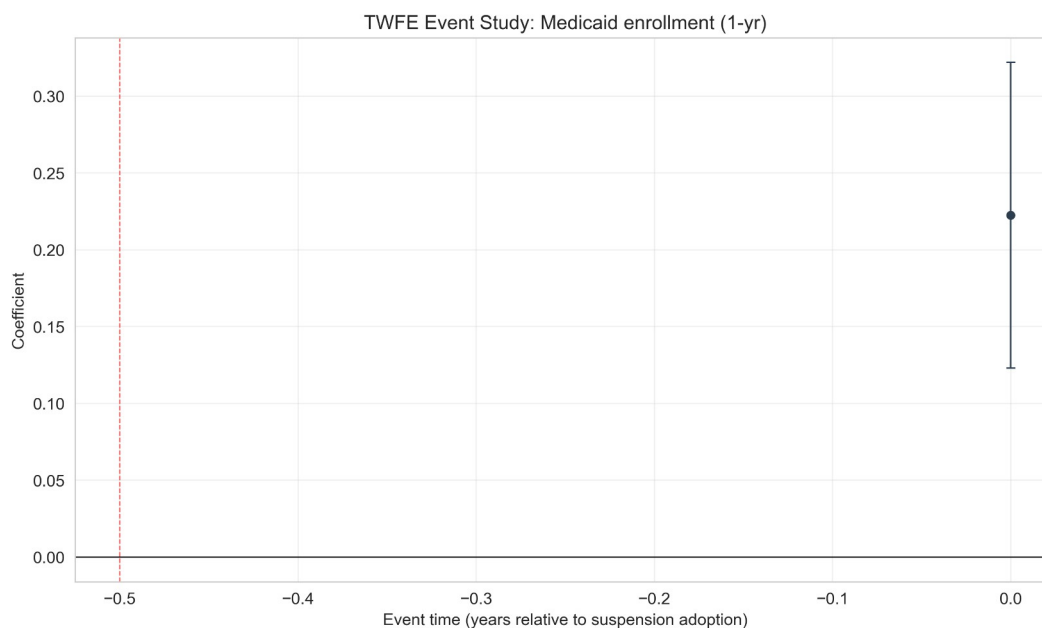


Figure 1

Figure 2. TWFE event study: W-2 employment, 1-year post-release.

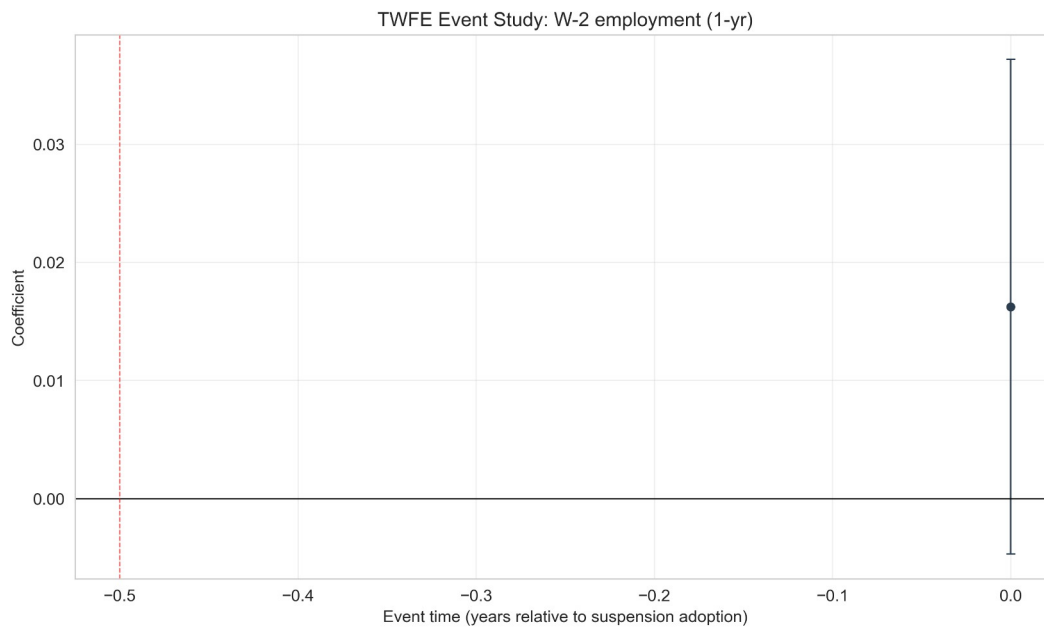


Figure 2

Figure 3. TWFE event study: W-2 earnings, 1-year post-release.

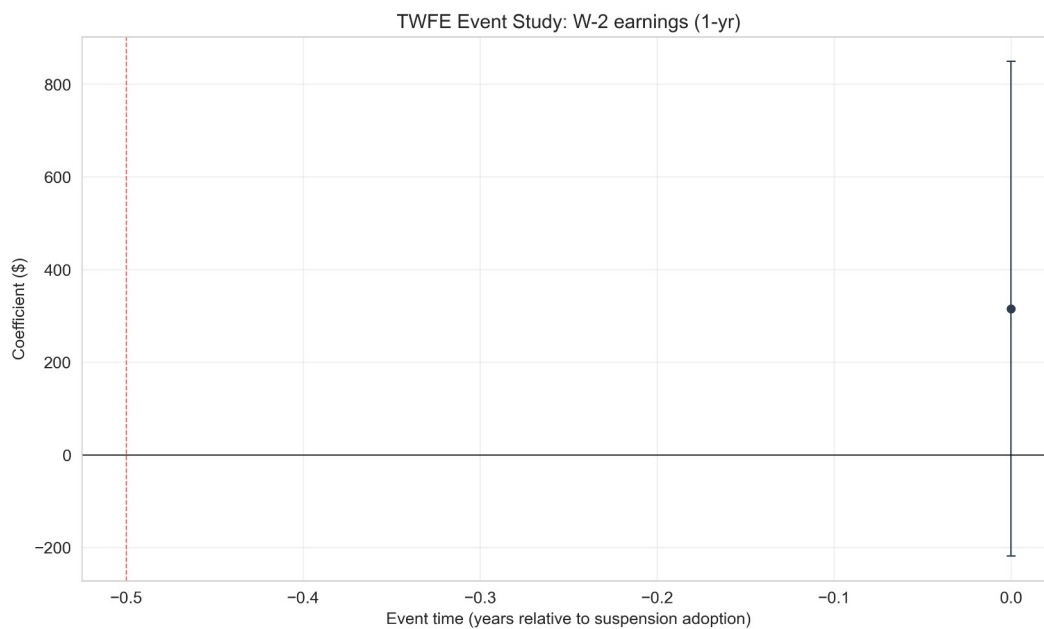


Figure 3

Figure 4. TWFE event study: mortality, 1-year post-release.

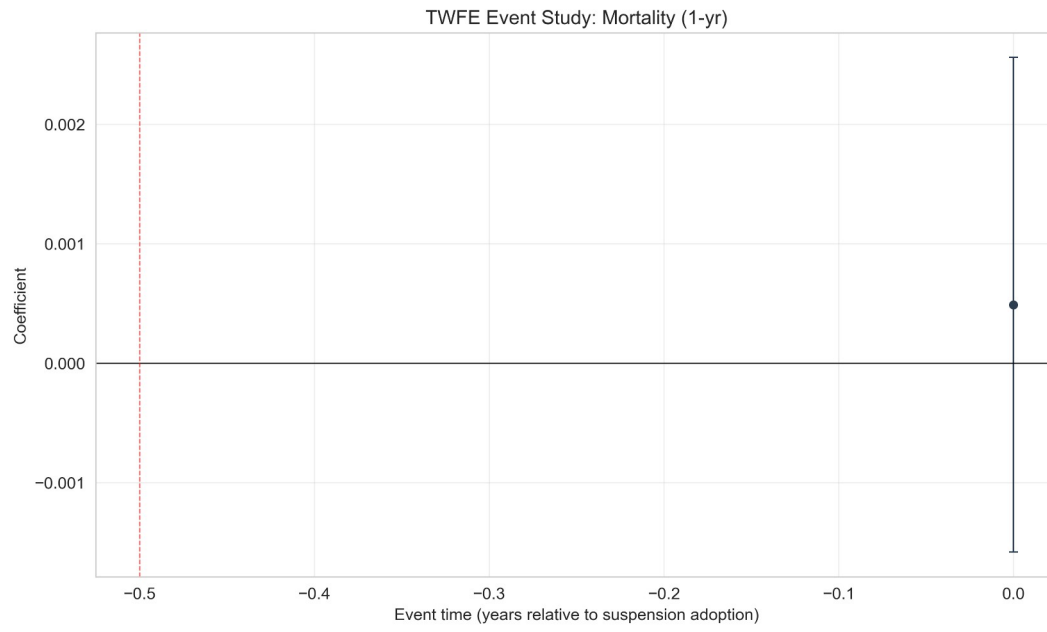


Figure 4